

Pacific Imaging Associates — Abdominal Ultrasound

RIGHT UPPER QUADRANT ULTRASOUND REPORT

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY — CONTAINS NO REAL PATIENT DATA

Patient: Diana R. Kowalczyk	DOB: 12/04/1983	MRN: 4417320
Date: November 5, 2024 at 11:00	Provider: Dr. Samuel Park, MD — Abdominal Radiology	

CLINICAL INDICATION

Ordering physician: Dr. T. Johnson, MD (General Surgery). Patient is a 41-year-old female with 6-month history of recurrent right upper quadrant abdominal pain, described as crampy and episodic, typically occurring 30-60 minutes after fatty meals and lasting 1-4 hours. Nausea present, no jaundice, normal liver function tests. Referred for evaluation of suspected cholelithiasis.

TECHNIQUE

Modality:	B-mode and Doppler ultrasound	Transducer:	Curved array 2-5 MHz
Preparation:	Patient fasted 4 hours prior to examination	Limitations:	Moderate body habitus; limited visualization of pancreatic tail

FINDINGS

GALLBLADDER: The gallbladder is moderately distended (8.4 x 3.2 cm). Multiple echogenic foci are identified within the gallbladder lumen, ranging in size from 3mm to 12mm, the largest measuring 1.2 cm in the gallbladder neck region. All demonstrate posterior acoustic shadowing, consistent with cholelithiasis. No gallbladder wall thickening (wall 2mm, normal). No pericholecystic fluid. No sonographic Murphy sign elicited during graded compression over the gallbladder.

BILE DUCTS: Common bile duct measures 4mm in caliber (normal). No intrahepatic biliary ductal dilation. No choledocholithiasis identified on this study (note: CBD stones are best evaluated by MRCP or EUS if clinically suspected).

LIVER: The liver is normal in size (span 14.2cm at MCL). Echotexture is homogeneous and normal, without focal lesions. No hepatic mass or suspicious hepatic lesion. Main portal vein patent with normal hepatopetal flow on Doppler.

SPLEEN: Normal in size (9.1 cm). No focal splenic abnormality.

PANCREAS: Head and body visualized — normal echotexture. Pancreatic tail obscured by overlying bowel gas.

KIDNEYS: Right kidney 10.8 cm, left kidney 10.4 cm. Normal cortical echogenicity and corticomedullary differentiation bilaterally. No hydronephrosis. No calculi identified.

FREE FLUID: No ascites.

IMPRESSION

- CHOLELITHIASIS:** Multiple gallstones present, largest 1.2 cm, without sonographic evidence of acute cholecystitis (no wall thickening, no pericholecystic fluid, negative Murphy sign).
- Normal liver, bile ducts, pancreas (partially visualized), spleen, and kidneys.

Clinical correlation recommended. Given symptomatic cholelithiasis with classic biliary colic, surgical consultation for cholecystectomy is suggested.

Report finalized by Dr. Samuel Park, MD, November 5, 2024 at 12:30.

This document contains entirely synthetic, AI-generated data for demonstration purposes only. No real patient, provider, or institutional information is represented.